What to Notice and Write Down While Mom Is in the Hospital or Rehab

Fifteen everyday things to notice on your visits, with no diagnosing.

You sit with Mom every afternoon, and you may not be sure what you should be noticing. This guide names fifteen things worth paying attention to on your visits and, for each, what to write down. Some are about Mom. Some are about the papers, dates and conversations that come your way.

Notice and record. Don't interpret. You see only part of her day, so “I haven't seen this” is a fair thing to write, and it becomes a question for the staff. What any of it means for her care or her coverage is for the people whose job that is. The everyday activities below follow the lists published by the Administration for Community Living and the Family Caregiver Alliance; the wording about what to write down is ours.

What changed

1 The story so far

NOTICE How each person describes what brought Mom in, and where the accounts differ.

WRITE DOWN What happened, in your own words, with the date and who told you each part.

2 Before and now [3]

NOTICE Things Mom did easily before the hospital that someone helps with now.

WRITE DOWN Each one on its own line, with “before” and “now” beside it.

Everyday activities

3 Washing and dressing [1]

NOTICE Whether she does it herself, needs things set out, gets a hand, or has it done for her.

WRITE DOWN Which of those you saw, and on which day.

4 Eating [1]

NOTICE The meal tray: what she opens and eats on her own, and where someone steps in.

WRITE DOWN What you saw at the meals you were there for.

5 Using the toilet [1]

NOTICE Whether she goes on her own, calls for help, or a staff member goes with her.

WRITE DOWN What you saw, with the day, and no commentary.

6 Getting up and getting around [1] [3]

NOTICE Getting out of the bed or a chair, and walking: what she holds onto, what she uses, who is beside her.

WRITE DOWN What she used and who helped. If you haven't seen her walk, write that down as a question for her therapist.

Medications, memory and time on her own

7 The medication list [2] [3]

NOTICE Whether you've been given a current list, and whether anyone has compared it with what she took before the hospital.

WRITE DOWN Where the list is, its date, and any change you were told about and by whom. Change nothing yourself; questions go to her doctor or pharmacist.

8 Memory and following the conversation [4]

NOTICE A question repeated, the day lost track of, an instruction from the staff that didn't stay with her.

WRITE DOWN What happened and when, not what it means. Everyone forgets things once in a while; if it worries you, tell her doctor or nurse.

9 Time on her own

NOTICE What happens when nobody is in the room, and what the staff say about someone staying with her.

WRITE DOWN What you saw, and what the staff said in their words.

Papers, dates and bills

10 Papers you're handed [2]

NOTICE Anything in writing: instructions, forms, notices, a medication list.

WRITE DOWN What it is, its date and who gave it to you. Keep the originals in one folder, and ask for instructions in writing if you were only told.

11 Dates people mention

NOTICE A discharge date, a meeting, a day when something “may change.”

WRITE DOWN The date, who said it, and whether it was confirmed or only mentioned in passing.

12 Words about coverage [5]

NOTICE Any mention of Medicare, insurance, coverage, or “skilled” care.

WRITE DOWN The exact words, who said them and when. Don't work out what they mean; ask the person who said them.

13 Bills and what you're already paying [2]

NOTICE Any bill, statement or request for payment, and anything the family already pays for her care.

WRITE DOWN Who it's from, the date and what it says it is for. Keep the original.

People

14 Who's who [2]

NOTICE The people you deal with: the discharge planner or social worker, the nurses, the therapists, the business office.

WRITE DOWN Each name, role and phone number, and what each one told you.

15 Who can help, and how much [2]

NOTICE Which relatives and friends are able to help, with what, and the limits each of you has.

WRITE DOWN Who, what and when, and the limits too. The discharge staff need to hear about those.

When a note isn't enough

If something about Mom looks new or sudden while you're with her, tell her nurse then; don't save it for your notes. If a paper, a date or a bill is unclear, ask the person who gave it to you, or the facility's social worker or business office, what it is and what it is for. Questions about Medicare coverage go to the facility, to Mom's plan or to Medicare. Questions about Medicaid go to your Medicaid planner.

What this guide doesn't do

It doesn't say what any of these mean, what kind of care Mom needs, whether she can go home, or what any paper or bill means for her coverage. The care team and the right professionals do that, and they do it better with your notes in front of them. Nothing here is medical, legal, financial or tax advice.

Ready for the next step?

You know what to pay attention to on your visits, and you have notes to show for it. The care interview walks you through Mom's situation area by area so nothing important is overlooked, and turns your answers into a Family Care Profile you can bring to any care conversation.

Create Your Free Family Care Profile ›

WHERE THIS COMES FROM

1. Administration for Community Living (LongTermCare.gov), What is Long-Term Care?
2. Family Caregiver Alliance, Hospital Discharge Planning: A Guide for Families and Caregivers
3. Administration for Community Living (LongTermCare.gov), Avoiding a Fall
4. MedlinePlus (National Library of Medicine), Memory | MedlinePlus
5. Medicare.gov (Centers for Medicare & Medicaid Services), Nursing Home Coverage